

Within- and between-year patterns of Seasonal Influenza and Respiratory Syncytial Virus and the potential for concurrent outbreaks

Yang Liu^{*✉1}, Kemin Zhu^{*2}, Ling Yin², Mark Jit³

¹Department of Infectious Disease Epidemiology, London School of Hygiene & Tropical Medicine, London WC1E 7HT, United Kingdom

²Shenzhen Institutes of Advanced Technology, Chinese Academy of Sciences, Shenzhen, China

³Department of Population Health, New York University, New York, NY, United States

*These authors contributed equally.

✉ Corresponding author, yang.liu@lshtm.ac.uk

Keywords: respiratory pathogens, influenza, RSV, seasonal dynamics, survival analysis, concurrent outbreaks, geographic variation

1 Introduction (659 words)

Influenza viruses and respiratory syncytial virus (RSV) are respiratory pathogens that impose a large disease burden worldwide, causing millions of infections and substantial morbidity and mortality, including hundreds of thousands of deaths each year [World Health Organization \(2024\)](#); [Paget et al. \(2019\)](#); [Li et al. \(2022\)](#); ?. In many temperate and some subtropical regions, the disease burden associated with these pathogens exhibits strong within-year cyclical patterns, often peaking in winter or cooler months [Bloom-Feshbach et al. \(2013\)](#); [Lowen and Steel \(2014\)](#); [Chen et al. \(2021\)](#). Such within-year patterns may be driven by environmental and climatic factors (e.g. temperature and humidity), human behaviours (e.g. school terms, holiday travel, and indoor crowding), and immune dynamics (e.g. waning and boosting of population immunity) [Lowen and Steel \(2014\)](#); [Susswein et al. \(2023\)](#); [Ewing et al. \(2017\)](#); [Eales et al. \(2025a\)](#). These within-year patterns vary by region and pathogen. Between-year cyclical patterns may also occur, driven by changes in circulating pathogens (e.g., antigenic drift [Roberts et al. \(2024\)](#), antagonistic or synergistic interactions between pathogens [Li et al. \(2025a\)](#)), demographics (e.g., population turnover [Roberts et al. \(2024\)](#)), environmental and climatic patterns (e.g., El Niño/La Niña [Chung et al. \(2025\)](#)), immune dynamics (e.g., waning

immunity [de Jong et al. \(2024\)](#)), and behaviours (e.g., more risk-averse behaviours following large outbreaks [Chen et al. \(2024\)](#)).

Within- and between-year temporal dynamics sometimes cause the circulation of these seasonal respiratory pathogens to peak concurrently. Such concurrent outbreaks of seasonal respiratory pathogens have been observed in many settings, such as in Europe, the United States, and China, most of which involve both the influenza virus and RSV [Rao et al. \(2023\)](#); [Li et al. \(2024\)](#); [Učakar et al. \(2013\)](#); [Shirreff et al. \(2024\)](#). These concurrent outbreaks pose substantial challenges to public health systems by increasing demand for healthcare resources and potentially overwhelming healthcare systems [Michelson et al. \(2024\)](#); [Mellor et al. \(2025\)](#); [Foulkes et al. \(2025\)](#). Therefore, to better support health systems' preparation for "winter pressures", it is crucial to understand the risk of concurrent respiratory virus outbreaks.

By far, most relevant research around the timing of seasonal respiratory pathogens has focused on deriving and understanding their within-year patterns. The between-year patterns, however, have rarely been considered due to data constraints. Such analyses require long-term surveillance data at fine spatial and temporal resolution to be simultaneously available for multiple pathogens. Coarse spatial resolution, such as administrative level one (e.g. provincial or state-level) aggregate data, often masks spatial variations in both within- and between-year patterns, and thus dampens out the potential signal for concurrent outbreaks. Furthermore, validating the role played by between-year patterns in a robust manner requires multiple study populations, which in turn requires the type of data mentioned above—already rare—to be available for multiple locations.

Statistical models are useful tools for understanding the role of within- and between-year variations in seasonal respiratory pathogens and for anticipating near-term trends in terms of when the greatest burden of these pathogens is expected to occur. Seasonal and near-term forecasting is well established for respiratory infections such as influenza, with common targets including season onset, peak timing, and peak intensity [Reich et al. \(2019\)](#); [Biggerstaff et al. \(2016\)](#). However, most existing predictive models focus on single-pathogen dynamics and are designed to forecast the health outcomes attributable to individual pathogens (e.g. the number of RSV hospitalisations expected in coming weeks). Here, we provide a complementary framework to provide seasonal-level forecasts for the probability of a concurrent outbreak and the possible temporal interval until the next concurrent outbreak. This addresses a different aspect of respiratory public health risks that has been understudied despite being important to health predicting the probability and timing of concurrent outbreaks remains comparatively underexplored. To develop the seasonal forecasting framework for concurrent outbreaks, we face two key technical challenges. First, methods designed for

identifying within-year cyclical components may mischaracterise between-year patterns as residual noises. Second, for most locations, concurrent outbreaks are rare events. With finite observational windows, it is challenging to distinguish between events that "will not occur" and events that "have not yet occurred"[Suresh et al. \(2022\)](#).

To address known technical challenges, we compiled city-level seasonal influenza and RSV surveillance time series covering more than five years from across mainland China. Using these data, we developed an analytical framework that combines time-series analysis and survival analysis to quantify the probability of a concurrent outbreak and the interval to the subsequent concurrent outbreak. Although the estimates we provide focus on seasonal influenza and RSV, our framework is widely adaptable to studying other respiratory pathogens with continuous measured data describing their disease burden.

2 Methods (1099 words)

2.1 Data Sources and Data Preprocessing

We assembled paired long-term surveillance data for RSV and seasonal influenza in mainland China based on a unified inclusion strategy. We strictly required continuous, year-indexed time series (excluding aggregated seasonal averages) with a minimum duration of five consecutive years, collected at the city level with monthly or weekly resolution. RSV datasets were identified by screening the extraction database of a recent systematic review [Guo et al. \(2024\)](#). For the identified cities with eligible RSV data, we assembled matching influenza time series using a hybrid approach: we first screened major influenza systematic reviews [Diamond et al. \(2022\)](#); [Li et al. \(2021\)](#), and subsequently conducted targeted PubMed searches for specific cities where review data were unavailable. When multiple datasets existed for a city, we prioritized those with longer duration and laboratory confirmation. Data presented in figures were digitized using WebPlotDigitizer. For analysis, raw surveillance metrics were natural log-transformed to stabilize variance, with zeros replaced by a small constant. Detailed search strategies, data sources, and surveillance definitions are provided in the Supplementary Material.

2.2 Identifying within- and between-year cycles

We used a smoothing-based algorithm to extract the within- and between-year cycles from the disease burden time series for seasonal influenza and RSV. The foundation of this algorithm is LOESS (Locally Estimated Scatterplot Smoothing). Seasonal-Trend decomposition

using LOESS (STL) is commonly used to extract within-year cycles in time-series data, assuming single internal cycles. The smoothing-based approach is numerically more flexible compared to other approaches that require researchers to make assumptions about expected mathematical forms (e.g. cosine models or cubic spline-based models) [Madaniyazi et al. \(2022\)](#). We used STL to extract the within-year cycles from the disease burden time series for seasonal influenza and RSV as a baseline model.

We then used MSTL (Multiple Seasonal-Trend decomposition using LOESS), which allows for more than one cycle. With MSTL, the original time series were decomposed into four components: within-year patterns (also available through baseline STL models), between-year patterns, long-term trends, and residuals. We assumed the within-year cycle lengths to be between 10 and 14 months and between-year cycle lengths to be between 18 and 36 months. The upper limit of the between-year cycle lengths was further constrained by the total lengths of the series. For time series that do not contain at least two full cycles of a given period length, MSTL does not test that between-year cycle length, as this is a requirement of the MSTL algorithm. In other words, for each city and pathogen combination, we tested up to 90 different model specifications (5 within-year cycle lengths * 18 between-year cycle lengths) using MSTL. Correspondingly, we presented the STL comparator models using 5 different model specifications (5 within-year cycle lengths).

Model selection (from 5 STL models and up to 90 MSTL models) was performed using Akaike Information Criterion (AIC). The optimal model for each city-pathogen combination was defined as having the minimum AIC value. Following standard practice, we considered models with $\Delta AIC < 7$ (where $\Delta AIC = AIC_i - AIC_{\min}$) as having comparable empirical support. The optimal models and their comparable models were all included in further analysis (i.e. forward simulation and concurrent outbreak identification). The lowest AIC estimates achievable using a single-cycle model (i.e. STL models) were compared against the lowest AIC estimates achievable using multi-cycle models (i.e. MSTL models) to assess whether the inclusion of a between-year cycle improves predictive performance. If the latter is lower, then we conclude that between-year patterns contribute to predicting the burden in the context of the city-pathogen combination. Bayesian Information Criterion (BIC) was also examined as a sensitivity analysis.

2.3 Forward simulations of outbreaks and concurrent outbreak identification

To assess the implications of within- and between-year patterns on future disease burden, we performed forward simulations based on observed disease burden history and the optimal

and their comparable models derived using the methods above. These forward simulations were performed for long timeframes to ensure that within- and between-year patterns estimated using different time series may be overlaid for a given city. Within- and between-year patterns were simply repeated in these forward simulations. The trend and residual component time series from the optimal STL/MSTL models were fitted using an Auto-Regressive Integrated Moving Average (ARIMA) model. This ARIMA model was then used to generate 1000 stochastic forward trajectories of the trend and residual components per model selected. Since the ARIMA model was fitted to a seasonally adjusted time series, the mean-reverting properties ensure that long-term forward simulations remain bounded. This stochastic simulation framework was developed to characterise the uncertainty around event probabilities, rather than providing deterministic forecasts of future disease activities. When a given city-pathogen combination has n optimal models, then $1000 \times n$ stochastic forward trajectories were generated. All forward simulations were performed based on their original time scale, as presented in the source literature.

Using the forward simulations of both seasonal influenza and RSV disease burden, we explore the possibility of concurrent outbreaks across different outbreak thresholds. The measure of seasonal influenza and RSV disease burden relied on different epidemiological measures depending on the city and health systems (e.g. attributable hospital admissions, test positive rates) and therefore cannot be directly compared on their original scale. Therefore, we rescaled these measures by decile, leaving the 1st and 10th deciles unbounded. Concurrent outbreaks were defined to occur when seasonal influenza and RSV disease burden simultaneously exceeded the 70th, 80th, or 90th percentiles of their observed disease burden history. A year is considered a concurrent outbreak year if the concurrent outbreak threshold was exceeded at least once in that year.

2.4 Characterising concurrent outbreak frequency using survival analysis

Inter-concurrent outbreak intervals were calculated as the time elapsed between successive concurrent outbreak years. Directly counting the intervals between concurrent outbreaks over-weights short intervals as they have more placement opportunities given the finite forward simulation window. It also introduces right-censoring issues when no concurrent outbreak or only one concurrent outbreak was observed in the forward simulation window. To address these issues, we reframed the interval detection as a time-to-event problem, and estimated the inter-concurrent outbreak intervals using Kaplan-Meier survival analysis, with left truncation and right-censoring. The survival curve was fitted for each city at all three

different concurrent outbreak thresholds defined above. With this setup, we can calculate inter-concurrent outbreak intervals. In the context of this study, we used the time by which 50% of the forward simulation paths ($1000 \times n$ models) are expected to experience their subsequent concurrent outbreak event as the inter-concurrent outbreak interval.

2.5 Software and code availability

All aforementioned methods and analyses were implemented in R (v4.2.2) using the following packages: `forecast`, `mstl`, and `survival`. All codes are available on the GitHub repository: https://github.com/yangclaraliu/respiratory_multi_pathogen_seasonality.

3 Results (1531 words)

3.1 Identified datasets and geographic distribution

Our search strategy yielded eight paired time series that met the inclusion criteria for both RSV and seasonal influenza surveillance. These datasets cover a diverse geographic range across mainland China, spanning both northern and southern epidemiological regions (Figure 1). The identified cities include three in Northern China—Beijing, Lanzhou (Gansu), and Xi'an (Shaanxi)—and five in Southern China—Wuhan (Hubei), Suzhou (Jiangsu), Wenzhou (Zhejiang), Guangzhou (Guangdong), and Yunfu (Guangdong).

The assembled time series provide continuous, multi-year observations for both pathogens within each location. The specific surveillance metrics varied by study site, reflecting the heterogeneity of local surveillance systems. The majority of datasets (15 out of 16 pathogen-city combinations) were based on laboratory-confirmed cases, utilizing diagnostic methods such as immunofluorescence assays (IFA/DFA) or PCR (Supplementary Table S1). The single exception was the influenza surveillance data for Xi'an, which was based on syndromic surveillance of daily influenza-like illness (ILI) case counts without virological confirmation. Despite differences in scale and metric definitions, all time series captured the relative temporal dynamics required to characterize the seasonal alignment and interannual variation of the two pathogens.

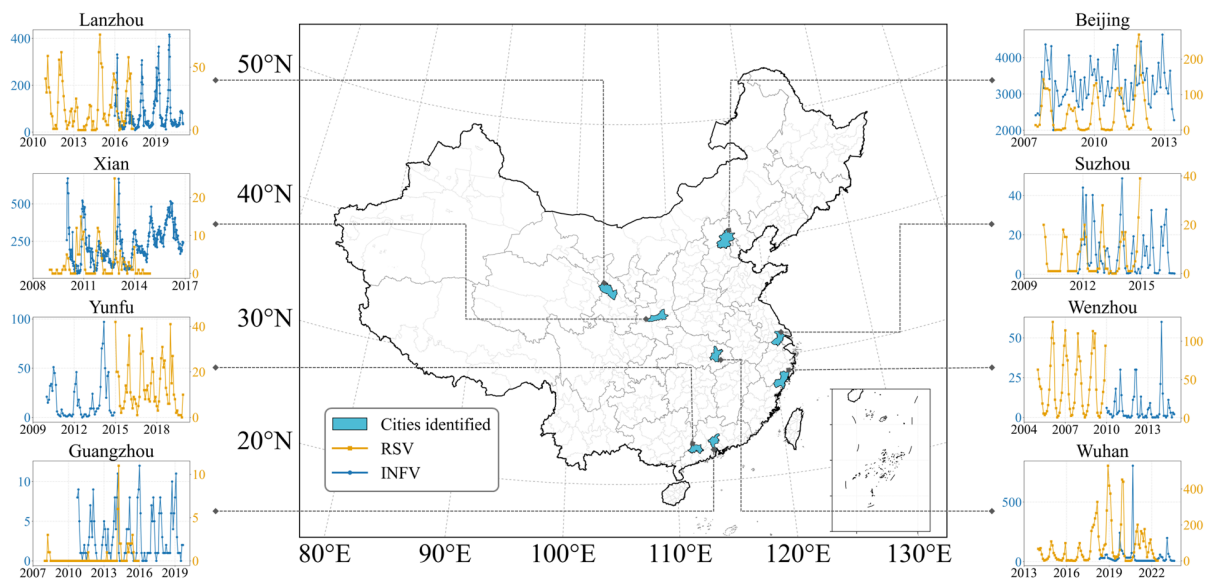


Figure 1: The geographic location of the eight study cities across mainland China and the historical epicurves of their seasonal influenza (green) and RSV (orange) disease burden.

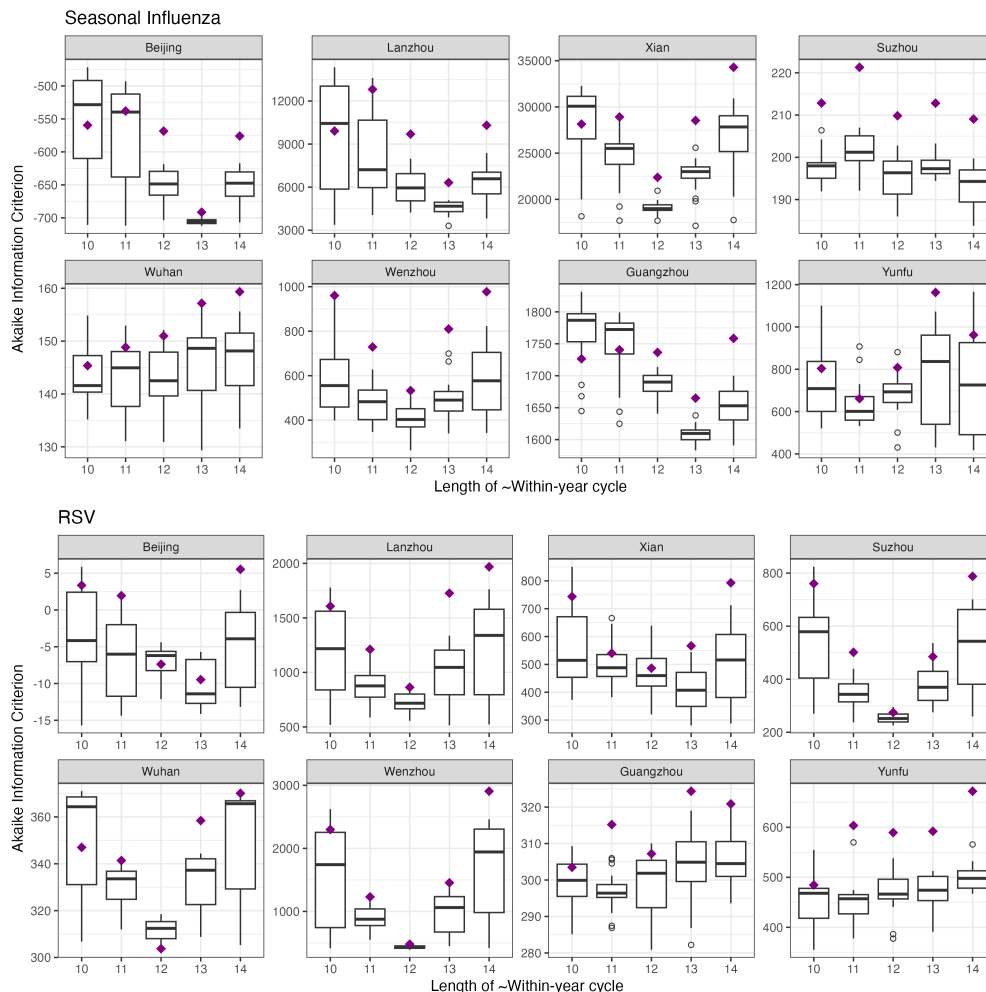


Figure 2: The association between within-year cycle lengths and Akaike Information Criterion (AIC) values by pathogen and city. Each purple point represents a model with only within-year cycles. Each Tukey boxplot represents the distribution of AIC values for models with the corresponding within-year cycle length and the full range of between-year cycle lengths tested.

We compared the performance of the models only adjusted for within-year cycles against the models adjusted for both within- and between-year cycles using AIC values (Figure 2). AIC values combine model fit (log-likelihood) with complexity penalties, and therefore the scales depend on both data characteristics and model structure. Therefore, AIC value comparisons are only meaningful if the underlying data set remains the same (i.e. the same city and pathogen combination). Lower AIC values indicate better model performance. In nearly all combinations of pathogen, city, and within-year cycle length, some models adjusted for both within- and between-year cycles performed better than the models only adjusted for within-

year cycles after penalising for increased degrees of freedom. The only exceptions were RSV in Wuhan - models with only annual cycles performed better than all other models tested. Thus, for seasonal influenza and RSV, significant between-year cycles may improve model performance beyond annual cycles alone.

For RSV, the relationship between within-year cycle length and AIC was often U-shaped, with the minimum at 12 months, indicating a best-fitting annual cycle across many city-pathogen combinations. In contrast, seasonal influenza showed a less consistent pattern: the AIC minimum frequently deviated from 12 months, implying that the optimal seasonal period is not strictly annual. This could reflect time-varying or heterogeneous periodicity (e.g., episodes of slightly shorter or longer cycles that average to ≈ 12 months over the long term) or unaccounted-for variation (e.g., due to factors not included in the model), despite long-term epidemiology appearing annual.

In several panels, the AIC values near the minimum were very densely distributed (i.e. small dispersion), indicating that several model specifications (from up to 90 MSTL model specifications) produce comparably performing models. Accordingly, in forward simulations and concurrent outbreak analysis, we carried forward both the optimal models and their comparable models ($\Delta\text{AIC} < 7$). The number of comparable models for influenza ranged from 2 (in Xi'an, Lanzhou, Wenzhou, and Yunfu) to 20 (in Wuhan and Beijing); that for RSV ranged from 2 (in Wenzhou and Yunfu) to 39 in Beijing. The full list detailing the number of comparable models by city and pathogen is available in the Supplementary Material (Table S1).

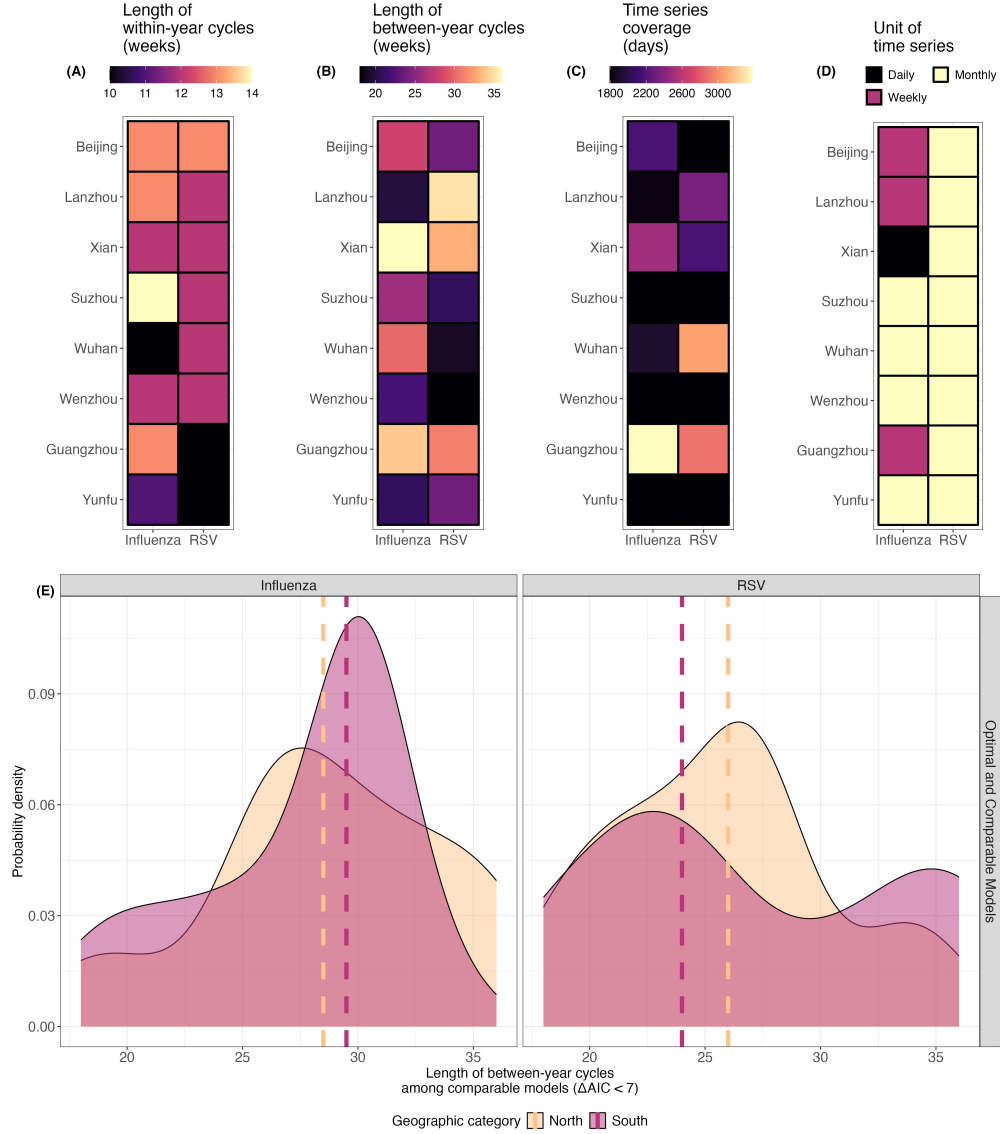


Figure 3: Seasonal component analysis and geographic patterns. (A) The optimal within-year cycle lengths estimated from MSTL models; (B) The optimal between-year cycle lengths estimated from MSTL models. As such, the corresponding cells in (A) and (B) belong to the same model specification; (C) The length of time series (converted to days to characterise temporal coverage); (D) The unit of time series (in their original scale); (E) Comparison of between-year cycle lengths among comparable models by geographic region (North vs South) and by pathogen (Influenza vs RSV).

The optimal within-year cycle lengths and between-year cycle lengths for each city and pathogen combination are presented in Figure 3 (A) and (B). These results reflect only the optimal models (the single best-performing model with lowest AIC per city-pathogen combination); their comparable models (those with $\Delta AIC < 7$) are not included. Similar to the results presented in Figure 2, most within-year cycles were approximately 12 months for

213 RSV and were not strictly annual for seasonal influenza. The between-year cycles selected for
 214 the optimal models were much more variable. The estimated lengths of within- and between-
 215 year cycles are not strongly associated with the length of the time series (Figure 3(C)) or
 216 their temporal resolution (Figure 3(D)). Looking across all optimal and their comparable
 217 models, the between-year cycles for influenza had a mean of 28 months in the North and 27
 218 months in the South; those for RSV had a median of 25 months in the North and 28 months
 219 in the South (Figure 3(E)). These differences are not statistically significant by region for
 220 either pathogen, which may be explained by small sample size and thus limited statistical
 221 power. The between-year cycles were highly variable (i.e. $IQR > 6$ months) for Suzhou
 222 (influenza), Wuhan (RSV) and Guangzhou (RSV) (see Supplementary Material Figure S1).

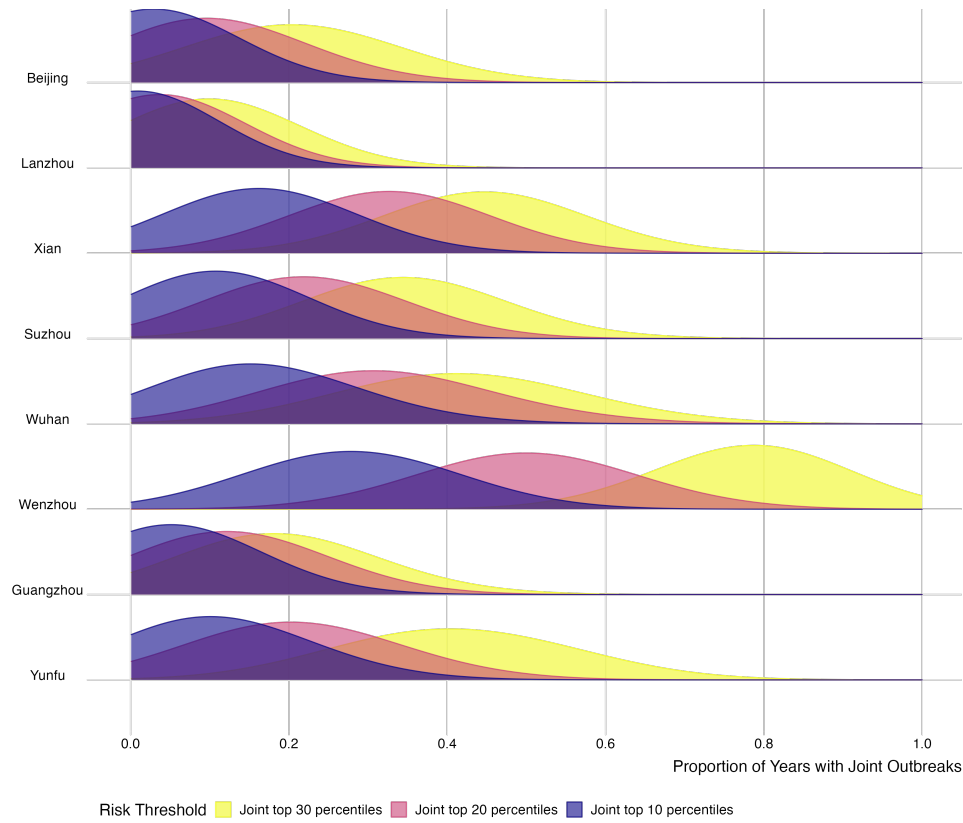


Figure 4: Proportion of years with concurrent outbreaks of seasonal influenza and RSV under different risk thresholds by city. Distributions are based on 1000 forward simulation trajectories using both the optimal and comparable models. Risk thresholds are defined by joint exceedance of the 70th, 80th, and 90th percentiles (corresponding to the top 30%, 20%, and 10%) of each city's observed disease burden history for both pathogens. Each ridge plot shows the density of the per-trajectory proportion of projected years with concurrent outbreaks (one proportion per simulation trajectory), calculated over the available projection horizon for each city-pathogen time series.

Forward projection not only reflects the within- and between-year cycles we estimated, but also the underlying city-specific epidemic histories. Using both optimal and comparable models, we demonstrate that there are non-negligible risks of concurrent outbreaks of influenza and RSV in the cities we investigated ([Figure 4](#)). Using the strictest risk threshold (joint top 10 percentile, i.e. simultaneously observing influenza and RSV exceeding the 90th percentile of the city’s respective observed history), Wenzhou was projected to experience the most frequent concurrent outbreaks (i.e. 25% of years projected), followed by Xi’an and Wuhan (15% of years projected). Beijing and Lanzhou were projected to not experience any concurrent outbreaks given this strict risk threshold. Under less strict thresholds, we found that the ranking by frequency remains consistent. When concurrent outbreak was defined by joint top 20 percentile (i.e. simultaneously observing influenza and RSV exceeding the 80th percentile of the city’s respective observed history), Beijing and Lanzhou were expected to observe some concurrent outbreaks (i.e. 10% and 5.9% of years projected, respectively). A complete table of median values of distributions presented in [Figure 4](#) can be found in the Supplementary Material Table S2.

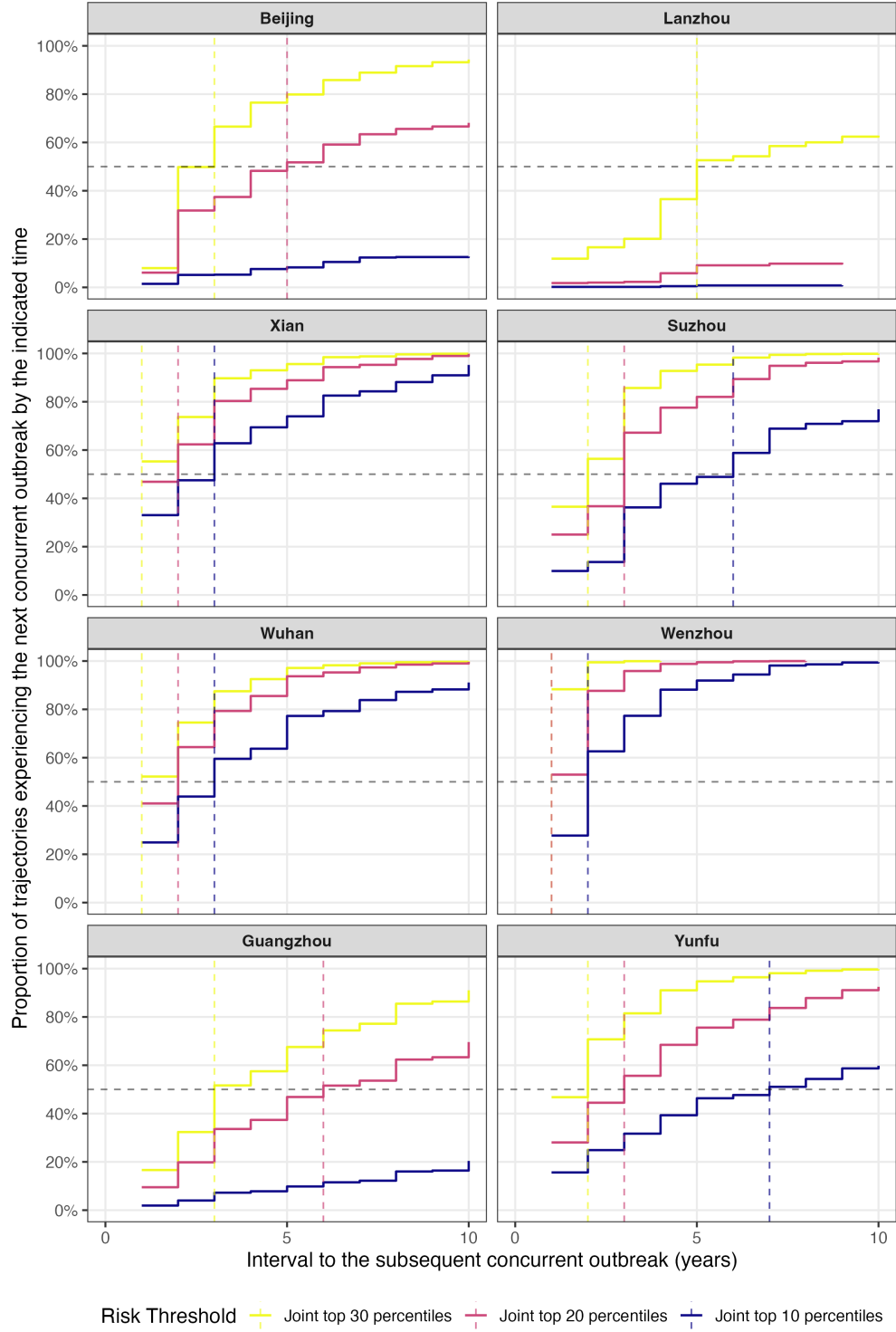


Figure 5: Cumulative probability of the next concurrent outbreak of seasonal influenza and RSV as a function of time since the most recent concurrent outbreak, by city. Curves are estimated from 1000 forward simulation trajectories using both optimal and comparable models. Risk thresholds are defined by joint exceedance of the 70th, 80th, and 90th percentiles (corresponding to the top 30%, 20%, and 10%) of each city's observed disease burden history for both pathogens. Vertical dashed lines indicate the median interval to the next concurrent outbreak for each threshold, and the horizontal dashed line marks the 50% probability level.

Using the predicted intervals between consecutive concurrent outbreak years from the forward simulations, we estimated the distribution of temporal intervals to the next potential concurrent outbreak for each city and threshold combination (Figure 6) using Kaplan-Meier survival analysis. Under the strictest risk threshold (joint top 10 percentile), the median temporal interval to the next concurrent outbreak was the shortest in Wenzhou, Xi'an, and Wuhan (two, three, and three years respectively). In other words, in these cities, at least 50% of the projection paths experienced another concurrent outbreak within two or three years after the last concurrent outbreak. In Suzhou and Yunfu, we expected longer temporal intervals between concurrent outbreaks (six and seven years, respectively). In Beijing, Lanzhou, and Guangzhou, concurrent outbreaks as defined by the strictest threshold were not likely based on the projection paths. Relaxing the joint epidemic threshold to the top 20 percentile (i.e. simultaneously observing influenza and RSV exceeding the 80th percentile of the city's respective observed history), the median temporal interval ranged from one to six years. Beijing and Guangzhou, which previously were not expecting any concurrent outbreaks, were now projected to experience them next in five or six years, respectively. Lanzhou was still projected to not experience any concurrent outbreaks given this outbreak risk threshold. A full table of the results can be found in the Supplementary Material Table S3.

4 Discussion (1559 words)

We used publicly available surveillance data on two major seasonal respiratory pathogens, seasonal influenza and respiratory syncytial virus (RSV), with at least 5 years of history from eight cities in mainland China to investigate within- and between-year patterns of their disease burden. Time-series analyses revealed that strong within-year cycles are present for both pathogens, consistent with the established understanding that they are fundamentally seasonal respiratory infections. Our model selection process further revealed that introducing between-year cycles can improve model fit, as measured by AIC, for both pathogens across nearly all cities (with the exception of RSV in Wuhan). We then combined time-series and survival analysis to quantify the probability of concurrent influenza–RSV outbreaks and the temporal intervals until the subsequent event of such nature under multiple risk thresholds. This analysis accounted for uncertainty around our models and the intrinsic stochasticity of the underlying data generation processes. We found that, for both pathogens, including between-year cycles improved model fit relative to within-year-only models. Concurrent outbreak risks varied substantially by city: under the strictest risk threshold, the median interval to the subsequent concurrent outbreak was 2–3 years in Wenzhou, Xi'an, and Wuhan,

whereas Beijing, Lanzhou, and Guangzhou did not experience concurrent outbreaks within the simulation window.

To our best knowledge, our study is the first to quantify the probabilities of concurrent outbreaks of seasonal respiratory pathogens and the temporal intervals until the subsequent concurrent outbreak under multiple risk thresholds. Most existing literature tends to focus on quantifying annual cycles. Only a small number of studies have quantified non-annual cycles (Yang et al., 2023; Li et al., 2025b) or confirmed the possibility of concurrent outbreaks of multiple seasonal respiratory pathogens (Pei and Shaman, 2020; Eales et al., 2025b). Therefore, a key point of novelty of our study is methodological. The survival analysis approach we incorporate into the predominant time-series-based theoretical framework allowed us to address two key technical challenges in outbreak prediction: finite sample bias and right censoring. An example of finite sample bias is that over a 10-year period, concurrent outbreaks one year apart may occur nine times, whereas concurrent outbreaks six years apart may only occur once. In the distribution of raw temporal intervals, a strong bias towards shorter intervals would occur. Right censoring occurs when we have to draw an end for the analytical window although the event of interest may not have happened yet. Without accounting for right censoring, an event that has "not yet happened" within the analytical window may be misinterpreted as an event that "would not happen". The relevance of this approach extends beyond influenza and RSV and can be broadly applicable to other respiratory diseases with continuously measured burden metrics.

In this study, concurrent outbreak risks were defined as the probability of simultaneously reaching the 70th, 80th, and 90th percentiles of historical values. By evaluating multiple thresholds, our framework accommodates different risk tolerance levels among decision makers—from those preparing for only the most severe concurrent outbreaks (90th percentile) to those planning for more frequent but less extreme events (70th percentile)—and quantifies how these choices affect estimated probabilities and recurrence intervals. This design enabled us to compare disease burden measures across cities and pathogens on a relative scale. This relative-scale design also reflects health system context—high health system pressure in Beijing and Yunfu would not look the same and cities should be evaluated relative to their own historical baseline.

We used a unique approach to time-series projection while assessing the probability of concurrent outbreaks and the temporal intervals to the subsequent concurrent outbreak, through which we propagate uncertainty from the model selection and data generation processes. As projections extend further into the future, uncertainty typically increases and can dominate the signal. To keep long-horizon results interpretable, our modelling framework regularises long-horizon uncertainty so that conclusions are driven by systematic dynamics

rather than accumulating stochastic noise. We therefore interpret results as seasonal-scale estimates of concurrent outbreak risk (i.e., the likelihood of co-circulation within a season), not as precise, year-specific predictions of when concurrent outbreaks will occur. This type of prediction is similar to those of some climate-sensitive infectious diseases that have been used in public health decision-making. Such a seasonal lens should complement real-time tools that forecast near-term incidence and operational public health endpoints in the context of respiratory public health management.

We found that RSV exhibited more consistently near-annual within-year cycles (i.e. around 12 months) than seasonal influenza, which is consistent with existing understanding of these pathogens' epidemiology. RSV likely shows a more consistent seasonal signal because its transmission relies more heavily on susceptibles from young age groups and experiences less intense competition between subtypes. However, given the relatively short observation window, distinguishing true deviations from an annual cycle is challenging. We therefore interpret our results as evidence of differences in the strength of seasonal regularity, rather than definitive proof of annual versus non-annual within-year cycles.

While between-year cycles for RSV have been observed elsewhere in the world, those of seasonal influenza have not been widely studied. Here, we estimated highly variable between-year cycles by city and by pathogen, indicating that the drivers of these cycles may be complex and context-specific, potentially driven by multiple slowly varying factors such as antigenic drift, viral interference, demographic turnover, behavioural changes, and immunisation history. Notably, the durations of within- and between-year cycles are not strongly associated with the probability of concurrent outbreaks or the temporal intervals to subsequent concurrent outbreaks. This indicates that predicting concurrent outbreaks requires consideration of not only pathogen characteristics and epidemiology but also the underlying transmission history.

Seasonal-level predictions of concurrent respiratory disease outbreaks are valuable for public health practitioners making early-season intervention decisions and for hospital system administrators seeking to prevent overcrowding and excessive strain on healthcare staff. Based on our estimates, communities in Xi'an, Wuhan, and Wenzhou may experience frequent concurrent outbreaks (i.e. temporal intervals to the subsequent concurrent outbreak are 2-3 years based on the strictest risk threshold) compared to other cities we investigated, and their local health systems may be better adapted to handling them. Our results provide evidence for the potential of further disease burden reduction if awareness of concurrent outbreaks is raised and early intervention strategies are implemented. In cities like Suzhou and Yunfu, however, concurrent outbreaks are less frequent (i.e. temporal intervals to the subsequent concurrent outbreak are 6-7 years based on the strictest risk threshold). At

lower frequencies, a concurrent outbreak may affect the community more severely, as baseline health systems are less prepared for such events. In these cities, seasonal-level concurrent outbreak predictions may be particularly useful when integrated with other real-time forecasting tools. Decisions may need to balance the trade-off between lower frequency and potentially larger negative impact.

There are, however, limitations to our study. First, we limited our analysis to influenza and RSV data from eight cities in mainland China. Our approach requires high-resolution surveillance data (i.e., city level) with long histories due to the nature of our analytical framework, which constrains the number of locations where this analysis can currently be applied. As new testing products that cover more pathogens become available, our approach will be applicable in more locations. While the approach we propose is broadly generalisable, the specific results are not. The cities we investigated are located in subtropical and temperate climates, where within-year cycles are strong for both influenza and RSV. This would not be the case in tropical regions, where seasonal patterns may be less pronounced or absent. Second, we used only one measure of disease burden per city and per pathogen. In practice, multiple measures of disease burden may be available (e.g., hospitalisations, deaths, outpatient visits, test positivity rates, etc.). Pooling all available information could improve our understanding of the underlying cycles and enhance prediction accuracy, but such harmonisation would require careful consideration of the underlying surveillance systems and their biases. Third, the mechanisms driving these cycles warrant further investigation. Finally, the longest between-year cycle we tested was 36 months, constrained by the available time-series data. Longer-term data made possible by electronic health systems may allow us to test for even longer cycles in the future.

Future research should explore several directions to enhance the practical utility of seasonal-level concurrent outbreak predictions. First, integration with real-time forecasting tools would provide more comprehensive situational awareness for public health decision-making. This integration would require incorporating real-time data streams, which introduce technical challenges such as backfilling and data quality control. Since concurrent outbreak risks are essentially a time-to-event problem, real-time systems would need to dynamically update predictions based on recent transmission history. For example, predictions should be adjusted when no concurrent outbreaks have occurred for an extended period (e.g. two years), reflecting the changing conditional probability as time since the last event increases. Second, prospective validation of our forward simulation approach against observed concurrent outbreaks would strengthen confidence in these predictions and help distinguish between the probabilistic assessments we provide and true point forecasts. Third, incorporating additional data sources could improve prediction accuracy. These might include

information about antagonistic or synergistic relationships between circulating pathogens (such as viral interference effects), changes in population immunity from vaccination campaigns, and shifts in pathogen antigenic characteristics that may influence the probability and timing of concurrent activity. Fourth, extending this framework to other combinations of co-circulating respiratory pathogens beyond influenza and RSV would test the generalisability of the approach and potentially reveal whether predictable patterns of concurrent circulation exist across broader pathogen communities.

References

- Biggerstaff, M., Alper, D., Dredze, M., Fox, S., Fung, I. C.-H., Hickmann, K. S., Lewis, B., Rosenfeld, R., Shaman, J., Tsou, M.-H., Velardi, P., Vespignani, A., Finelli, L., and Influenza Forecasting Contest Working Group (2016). Results from the centers for disease control and prevention’s predict the 2013-2014 influenza season challenge. *BMC Infectious Diseases*, 16:357.
- Bloom-Feshbach, K., Alonso, W. J., Charu, V., Tamerius, J., Simonsen, L., Miller, M. A., and Viboud, C. (2013). Latitudinal variations in seasonal activity of influenza and respiratory syncytial virus (rsv): a global comparative review. *PLoS ONE*, 8(2):e54445.
- Chen, C., Zhang, X., Jiang, D., Yan, D., Guan, Z., Zhou, Y., Liu, X., Huang, C., Ding, C., Lan, L., Huang, X., Li, L., and Yang, S. (2021). Associations between temperature and influenza activity: A national time series study in china. *International Journal of Environmental Research and Public Health*, 18(20):10846.
- Chen, Z., Tsui, J. L., Gutierrez, B., Busch Moreno, S., du Plessis, L., Deng, X., Cai, J., Bajaj, S., Suchard, M. A., Pybus, O. G., Lemey, P., Kraemer, M. U. G., and Yu, H. (2024). Covid-19 pandemic interventions reshaped the global dispersal of seasonal influenza viruses. *Science*, 386(6722):eadq3003. Epub: 2024-11-08.
- Chung, M. V., Vecchi, G. A., Yang, W., Grenfell, B., and Metcalf, C. J. E. (2025). Intersecting memories of immunity and climate: Potential multiyear impacts of the El Niño–Southern Oscillation on infectious disease spread. *GeoHealth*, 9(2):e2024GH001193. Published: 2025-02-11.
- de Jong, S. P. J., Felix Garza, Z. C., Gibson, J. C., et al. (2024). Determinants of epidemic size and the impacts of lulls in seasonal influenza virus circulation. *Nature Communications*, 15:591. Published: 2024-01-18.

411 Diamond, C., Gong, H., Sun, F. Y., Liu, Y., Quilty, B. J., Jit, M., Yang, J., Yu, H., Ed-
412 munds, W. J., and Baguelin, M. (2022). Regional-based within-year seasonal variations in
413 influenza-related health outcomes across mainland china: a systematic review and spatio-
414 temporal analysis. *BMC Medicine*, 20(1):58.

415 Eales, O., Shearer, F. M., and McCaw, J. M. (2025a). How immunity shapes the long-term
416 dynamics of influenza h3n2. *PLOS Computational Biology*, 21(3):e1012893.

417 Eales, O., Windecker, S. M., McCaw, J. M., and Shearer, F. M. (2025b). Inferring temporal
418 trends of multiple pathogens, variants, subtypes or serotypes from routine surveillance
419 data. *American Journal of Epidemiology*.

420 Ewing, A., Lee, E. C., Viboud, C., and Bansal, S. (2017). Contact, travel, and transmission:
421 The impact of winter holidays on influenza dynamics in the united states. *Journal of*
422 *Infectious Diseases*, 215(5):732–739.

423 Foulkes, S., Munro, K., Sparkes, D., Khawam, J., Russell, S., Dunne, A., Timeyin, J.,
424 Hettiarachchi, N., Cairns, M., Bradley, D. T., De Lacy, E., Wilson, K., Andrews, N.,
425 Charlett, A., Respiratory Virus and Microbiome Initiative (RVI) team, Bellis, K., Huang,
426 Y.-L., Forbes, M., Frick-Kretschmer, A., Knoll, M., Harrison, E. M., SIREN study group,
427 Brown, C. S., Atti, A., Islam, J., Hopkins, S., and Hall, V. (2025). Prevalence and impact
428 of sars-cov-2, influenza, respiratory syncytial virus (rsv) infection and respiratory illness
429 on uk healthcare workers during winter 2023/24 (september 2023 to march 2024): Siren
430 cohort study. *Journal of Infection*, 91(4):106620. Epub 2025 Oct 6.

431 Guo, L., Deng, S., Sun, S., Wang, X., and Li, Y. (2024). Respiratory syncytial virus sea-
432 sonality, transmission zones, and implications for seasonal prevention strategy in china: a
433 systematic analysis. *The Lancet Global Health*, 12(6):e1005–e1016.

434 Li, J., Chen, Y., Wang, X., and Yu, H. (2021). Influenza-associated disease burden in
435 mainland china: a systematic review and meta-analysis. *Scientific Reports*, 11(1):2886.

436 Li, K., Hamrin, J., Nilsson, A., et al. (2025a). Unraveling the role of viral interference
437 in disrupting biennial RSV epidemics in northern stockholm. *Nature Communications*,
438 16:8137. Published: 2025-08-30.

439 Li, K., Hamrin, J., Nilsson, A., Weinberger, D. M., and Pitzer, V. E. (2025b). Unraveling
440 the role of viral interference in disrupting biennial rsv epidemics in northern stockholm.
441 *Nature Communications*, 16(1):8137.

- Li, Y., Wang, X., Blau, D. M., Caballero, M. T., Feikin, D. R., Gill, C. J., Madhi, S. A., Omer, S. B., Simões, E. A. F., Campbell, H., Pariente, A. B., Bardach, A., Bassat, Q., Casalegno, J.-S., Chakhunashvili, G., Crawford, N., Danilenko, D., Do, L. A. H., Echavarria, M., Gentile, A., Gordon, A., Heikkinen, T., Huang, Q. S., Jullien, S., Krishnan, A., Lopez, E. L., Markić, J., Mira-Iglesias, A., Moore, H. C., Moyes, J., Mwananyanda, L., Nokes, D. J., Noordeen, F., Obodai, E., Palani, N., Romero, C., Salimi, V., Satav, A., Seo, E. H., Shchomak, Z., Singleton, R., Stolyarov, K., Stoszek, S. K., von Gottberg, A., Wurzel, D., Yoshida, L.-M., Yung, C.-F., Zar, H. J., Respiratory Virus Global Epidemiology Network, Nair, H., and RESCEU investigators (2022). Global, regional, and national disease burden estimates of acute lower respiratory infections due to respiratory syncytial virus in children younger than 5 years in 2019: a systematic analysis. *The Lancet*, 399(10340):2047–2064.
- Li, Y., Yu, J., Wang, Y., Yi, J., Guo, L., Wang, Q., Zhang, G., Xu, Y., and Zhao, Y. (2024). Cocirculation and coinfection of multiple respiratory viruses during autumn and winter seasons of 2023 in beijing, china: A retrospective study. *Journal of Medical Virology*, 96(4):e29602.
- Lowen, A. C. and Steel, J. (2014). Roles of humidity and temperature in shaping influenza seasonality. *Journal of Virology*, 88(14):7692–7695.
- Madaniyazi, L., Tobias, A., Kim, Y., Chung, Y., Armstrong, B., and Hashizume, M. (2022). Assessing seasonality and the role of its potential drivers in environmental epidemiology: a tutorial. *International journal of epidemiology*, 51(5):1677–1686.
- Mellor, J., Tang, M. L., Jones, O., Ward, T., Riley, S., and Deeny, S. R. (2025). Forecasting covid-19, influenza, and rsv hospitalizations over winter 2023-4 in england. *International Journal of Epidemiology*, 54(3):dyaf066.
- Michelson, K. A., Ramgopal, S., Kociolek, L. K., Zerr, D. M., Neuman, M. I., Bettenhausen, J. L., Hall, M., and Macy, M. L. (2024). Children’s hospital resource utilization during the 2022 viral respiratory surge. *Pediatrics*, 154(1):e2024065974.
- Paget, J., Spreuwenberg, P., Charu, V., Taylor, R. J., Iuliano, A. D., Bresee, J., Simonsen, L., Viboud, C., associated Mortality Collaborator Network, G. S. I., and Teams, G. C. (2019). Global mortality associated with seasonal influenza epidemics: New burden estimates and predictors from the glamor project. *Journal of Global Health*, 9(2):020421.
- Pei, S. and Shaman, J. (2020). Aggregating forecasts of multiple respiratory pathogens

supports more accurate forecasting of influenza-like illness. *PLOS Computational Biology*, 16(10):e1008301.

Rao, S., Armistead, I., Tyler, A., Lensing, M., Dominguez, S. R., and Alden, N. B. (2023). Respiratory syncytial virus, influenza, and coronavirus disease 2019 hospitalizations in children in colorado during the 2021-2022 respiratory virus season. *The Journal of Pediatrics*, 260:113491. Article number: 113491.

Reich, N. G., Brooks, L. C., Fox, S. J., Kandula, S., McGowan, C. J., Moore, E., Osthus, D., Ray, E. L., Tushar, A., Yamana, T. K., Biggerstaff, M., Johansson, M. A., Rosenfeld, R., and Shaman, J. (2019). A collaborative multiyear, multimodel assessment of seasonal influenza forecasting in the united states. *Proceedings of the National Academy of Sciences of the United States of America*, 116(8):3146–3154.

Roberts, M. G., Hickson, R. I., and McCaw, J. M. (2024). How immune dynamics shape multi-season epidemics: a continuous-discrete model in one dimensional antigenic space. *Journal of Mathematical Biology*, 88:48. Published: 2024-03-27.

Shirreff, G., Chaves, S. S., Coudeville, L., Mengual-Chuliá, B., Mira-Iglesias, A., Puig-Barberá, J., Orrico-Sanchez, A., Díez-Domingo, J., Valencia Hospital Surveillance Network for the Study of Influenza and Other Respiratory Viruses (VAHNSI), Opatowski, L., and Lopez-Labrador, F. X. (2024). Seasonality and co-detection of respiratory viral infections among hospitalised patients admitted with acute respiratory illness-valencia region, spain, 2010-2021. *Influenza and Other Respiratory Viruses*, 18(10):e70017.

Suresh, K., Severn, C., and Ghosh, D. (2022). Survival prediction models: an introduction to discrete-time modeling. *BMC Medical Research Methodology*, 22(1):207.

Susswein, Z., Rest, E. C., and Bansal, S. (2023). Disentangling the rhythms of human activity in the built environment for airborne transmission risk: An analysis of large-scale mobility data. *eLife*, 12:e80466.

Učakar, V., Sočan, M., and Prosenc Trilar, K. (2013). The impact of influenza and respiratory syncytial virus on hospitalizations for lower respiratory tract infections in young children: Slovenia, 2006-2011. *Influenza and Other Respiratory Viruses*, 7(6):1093–1102.

World Health Organization (2024). Influenza (seasonal). [https://www.who.int/news-room/fact-sheets/detail/influenza-\(seasonal\)](https://www.who.int/news-room/fact-sheets/detail/influenza-(seasonal)). Accessed: 2026-01-23.

Yang, F., Servadio, J. L., Le Thanh, N. T., Lam, H. M., Choisy, M., Thai, P. Q., Nhu Thao, T. T., Thao Vy, N. H., Phuong, H. T., Nguyen, T. D., Hoai Tam, D. T., Hanks, E. M.,

506 Vinh, H., Bjornstad, O. N., Van Vinh Chau, N., and Boni, M. F. (2023). A combination of
507 annual and nonannual forces drive respiratory disease in the tropics. *BMJ Global Health*,
508 8(11):e013054.